

EVIDENCE LOG

For Tinnitus (Ringing in Ears) VA Disability Claims

THE SMOKING GUN EVIDENCE

After years of navigating the VA claims process—as veterans ourselves—we discovered something that changes everything: **the single most powerful piece of evidence you can submit isn't officially part of a Fully Developed Claim.**

"No more convincing. Just corroboration."

When you walk into a C&P exam without this documentation, the examiner's job is to **investigate whether your condition exists and how severe it is.** They're playing detective. You're trying to convince them.

When you walk in WITH a properly completed Evidence Log, the dynamic completely shifts. The examiner's job becomes **corroborating what you've already documented.** They're not investigating—they're verifying. That's a fundamentally different exam.

HOW VA RATERS ACTUALLY EVALUATE YOUR CLAIM

Every VA disability rating—regardless of condition—is based on three core factors. Raters are trained to look for evidence of these specific elements:

FACTOR	WHAT IT MEANS	WHAT RATERS LOOK FOR
FREQUENCY	How often do your symptoms occur?	Constant? Intermittent? Nighttime only?
SEVERITY	How intense are your symptoms when they occur?	Volume level? Distracting?
FUNCTIONAL IMPACT	How do symptoms affect your daily life?	Sleep (Insomnia)? Concentration? Irritability?

Why This Evidence Log Works: Tinnitus is subjective—only you can hear it. A log proves it is "Recurrent" (persistent) and documents the *secondary* impacts (like anxiety or sleep apnea connection) which are often crucial for higher aggregate ratings.

DO NOT FILE ANY CLAIM UNTIL THIS LOG IS COMPLETED AND INCLUDED WITH YOUR SUBMISSION

TINNITUS: CONDITION-SPECIFIC GUIDANCE

Tinnitus is rated under **38 CFR § 4.87, Diagnostic Code 6260**. The maximum schedular rating for tinnitus is usually **10%**. There is no 30% or 50% for tinnitus alone.

Why Log It?

1. To prove "Recurrent" status (required for the 10%).
2. To build evidence for **Secondary Conditions** (e.g., Tinnitus causing Insomnia, Anxiety, or Migraines).
3. To document the "Functional Impact" for a narrative statement.

What C&P Examiners Are Trained to Document:

- **Recurrence:** Is it constant or does it come and go?
- **Etiology:** Noise exposure in service?
- **Impact:** Does it affect sleep? Concentration? Hearing conversations?

Tinnitus Rating Levels:

RATING	LEVEL OF IMPAIRMENT	SYMPTOMS TYPICALLY PRESENT
10%	Recurrent Tinnitus	Persistent ringing, buzzing, hissing, or clicking in the ears.

How to Use the Evidence Log for Tinnitus:

Focus on the *impact*. Since the sound is internal, describe how it changes your behavior.

Strong Entry Example: "High-pitched ringing started 0800. Volume 7/10. Could not focus on reading report at work—had to use white noise machine. Ringing spiked at 2200 while trying to sleep. Tossed and turned for 2 hours. Anxiety increased due to noise."

Weak Entry Example: "Ears ringing today."

COMPLETED EXAMPLE: 7-DAY EVIDENCE LOG

Below is an example of a properly completed Evidence Log.

DATE	TIME	SYMPTOMS & SEVERITY	FUNCTIONAL IMPACT
Mon 1/6	All day	Low hum (3/10) during day. Spiked to high pitch (8/10) at 2100 in quiet room.	Difficulty falling asleep (took 1 hr). Had to turn on TV to drown out noise.
Tue 1/7	1000-1400	Loud ringing in left ear (6/10) after loud noise at construction site near office.	Distracted at work. Missed part of phone conversation. Irritable.
Wed 1/8	All day	Constant high pitch. 5/10.	Hard to hear wife over dinner. Felt isolated.
Thu 1/9	Night	Ringing severe (9/10) at bedtime.	Insomnia. Slept only 4 hours. Woke up fatigued.
Fri 1/10	All day	Moderate ringing 4/10.	Used headphones with background noise to focus.
Sat 1/11	-	Mild ringing 2/10.	Tolerable.
Sun 1/12	2000-EOD	Screeching noise in both ears 8/10.	Could not read book. Agitated. Snapped at family due to frustration.

YOUR EVIDENCE LOG: 30-DAY TEMPLATE

Instructions: Complete one row per day for at least 30 consecutive days.

VETERAN NAME: _____ **CLAIM #:** _____
CONDITION: Tinnitus (DC 6260) **START DATE:** _____

DATE	TIME	SYMPTOMS & SEVERITY (Volume, pitch, ear?)	FUNCTIONAL IMPACT (Sleep, concentration, irritability?)
Day 1			

Day 2			
Day 3			
...			
Day 30			

Note: Print this page multiple times to cover 30 days.